SAMPLE ANSWER · STROKE · CASE 4

Sample 7-minute ISBAR: George

George Haddad · 72-year-old man · Acute stroke unit, public hospital · about 999 spoken words (≈ 7.1 min at 140 words per minute)

I Introduction

60 words

"Hi, I'm the occupational therapy student who came to review George Haddad's right arm positioning. George is 72 and is four days after a left hemisphere ischaemic stroke. I need to raise an urgent concern: he's being given food and fluids that don't match his swallowing recommendations, and he's showing signs that they may be going down the wrong way."

S Situation

110 words

"When I arrived just after midday, George was semi-reclined and slumped to the right. His daughter Rania was feeding him a meat pie from the café and regular water from his jug. He coughed after the pie and again after the water, and when he tried to speak his voice sounded wet and gurgly. He's also been sleepy all morning.

His right arm was hanging off the bed, his right hand looks puffy, and he grimaced when I moved the arm. His nurse is with another patient, Rania is about to give him more pie, and his wife Mary has just arrived with home-made soup. I need your help now."

B Background

148 words

"George has dense right-sided weakness and aphasia; he understands simple sentences but can say only a few words. Speech pathology assessed his swallow on day two and recommended IDDSI Level 5 minced and moist food and Level 2 mildly thick fluids, sitting fully upright with supervision for all food and drinks. There's a sign above his bed, but a get-well card is covering it. Speech pathology plans to review him tomorrow.

Nursing notes say he ate little breakfast, and the family are unhappy with the hospital food. He lives with Mary in a two-storey townhouse, and Rania visits daily. Before the stroke he was independent and loved his vegetable garden.

I couldn't find observations since this morning, including temperature and oxygen saturation. It's not documented whether the family have been told about the swallowing recommendations, which medications he's on, or whether his shoulder has been assessed for pain."

A Assessment

330 words

"My most urgent concern is aspiration. George was assessed as needing modified food and thickened fluids, and today he has had regular pastry, meat and thin water. Coughing during eating and drinking and a wet-sounding voice are warning signs that food or fluid may be entering his airway. Several other things increase that risk. He's slumped, not upright. He's drowsy, so his ability to protect his airway may be reduced. And he can't easily tell anyone he's struggling because of his aphasia. Aspiration can lead to choking or pneumonia, which could set back his recovery significantly.

His drowsiness also needs attention in its own right. Rania thinks it's 'the tablets', and that may be true, but I can't assume that four days after a stroke. It could reflect medication, a developing chest infection or a change in his neurological status, so it needs medical review.

My second concern is George's right shoulder. A heavy, weak arm hanging off the bed puts traction on the shoulder joint, and his grimace and puffy hand suggest pain and swelling. That increases the risk of ongoing shoulder pain, which can limit his rehabilitation. This is the task I came to do, and it matters, but it comes after his airway.

I also want to acknowledge the family. Their distress about him not eating is real, and they're feeding him out of love. But the coughing isn't because 'the pastry's dry'. I think they haven't understood why the recommendations matter. Rania's questions about the aged care assessment, and Mary wanting him home, are important conversations, but it's far too early in his recovery to plan his discharge destination now. And the TV remote can wait.

So my priority is: immediately, stop oral intake and sit him upright; then nursing, medical and speech pathology review; then shoulder protection; and later, family education and discharge planning. As a student, I won't assess his swallow or chest myself. My role is to recognise the risk and escalate it."

R Recommendation

351 words

"First, I'd calmly ask Rania and Mary to pause the pie, water and soup for now, explaining that George's swallow needs special textures and he's coughing. I'd sit him fully upright with his trunk supported in midline, and use the call bell to get his nurse straight away. If he struggles to breathe or his colour changes, I'd call for emergency help immediately.

Second, I'd like your help to escalate today. I'd ask nursing to check his observations, including temperature, oxygen saturation and breathing. I'd ask the medical team to review him for possible aspiration and to look at his drowsiness, including a medication review with pharmacy. I'd ask speech pathology to review him today rather than tomorrow if possible, and to talk with the family about why the recommendations matter. A dietitian could look at his poor intake and help find modified versions of foods he enjoys, which might ease the family's worries. I'd also uncover the sign.

Third, for his shoulder, I'd support the arm on a pillow, elevate the hand and ask nursing to avoid pulling on the arm during handling. I'd then complete my positioning review: a structured observation of his positioning in bed and in the chair, with a shoulder check for pain, subluxation and swelling. That's my principal short-term assessment. It fits the Stroke Foundation Living Clinical Guidelines, which recommend shoulder protection and positioning to reduce shoulder pain after stroke. I'd then write a positioning plan with physiotherapy and nursing.

For the longer term, I'd use the Modified Barthel Index to track his self-care and mobility over his admission. That gives the team and family objective information when it is time to think about where he goes.

Social work could meet the family later about the aged care assessment and Mary's wish to have him home, once his recovery is clearer. I'd involve George in decisions using yes/no questions and aphasia-friendly communication, with speech pathology's advice, and ask about his gardening and family goals.

> I'll document what I observed and who I informed, then follow up after speech pathology review to complete the positioning plan."

Coaching notes *not spoken*

The immediate priority

George is coughing with a wet voice while being given regular food and thin fluids against his IDDSI 5 and Level 2 recommendations, while slumped and drowsy. Stop oral intake, sit him upright, get his nurse and escalate to speech pathology and medical. The shoulder comes second.

Distractors in this case

- **The arm positioning referral** — the task you came for, but airway first.
- **"He's just coughing because the pastry's dry"** — family reassurance that doesn't fit the signs.
- **"Sleepy because of the tablets"** — may be true, but drowsiness also needs review.
- **Aged care assessment and Mary wanting him home** — important, but too early four days post-stroke.
- **TV remote; "won't eat hospital food"** — tempting to help, but not the priority.

Missing information to clarify

- Observations since this morning (temperature, oxygen saturation, respiratory rate)
- Whether the family received swallowing education
- Current medications and any sedating effects
- Whether the shoulder has been assessed for pain or subluxation

Useful references

- International Dysphagia Diet Standardisation Initiative (IDDSI) framework
- Stroke Foundation, *Living Clinical Guidelines for Stroke Management* (dysphagia; shoulder pain and positioning)

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